



The Hospital Sentence Bank

30 exact sentences for the hard moments · from Shelbi Kohler, certified birth doula ·
truejoybirthing.com

True Joy Birthing provides birth education, not medical advice. These sentences are communication tools to help you talk with your care team — they are not a substitute for your provider's guidance. Your care team knows your unique situation best; always follow their recommendations for medical decisions.

How to use this

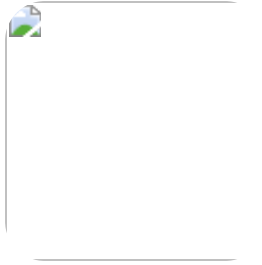
1 • Save it to your phone. Screenshot the cards you might need — or keep this PDF in your camera roll. The cards are built to be readable at a glance, one sentence each.

2 • Find your moment. When something unexpected comes up, flip to the category that matches what's happening. Each category opener tells you exactly when it applies.

3 • Say the sentence — out loud, or hand the card to your person. You don't need to improvise. Every sentence is polite, firm, and collaborative. It opens the conversation; it never picks a fight.

4 • Then ask "what happens next?" Every card includes it — the questions that keep you informed as things move.

You've got this. — Shelbi



1 • Induction Pressure

5 sentences for this moment.

INDUCTION PRESSURE



“Can you walk me through why you’re recommending induction today, and what my alternatives are?”

When it comes up: your provider suggests starting labor before it starts on its own — at a checkup, over the phone, or at the hospital.

Why it works: it asks for the *reason* and the *options* in one calm breath. It tells your team you're paying attention without challenging their judgment, and it buys you a real conversation instead of a yes.

What happens next: you'll get the medical picture (your baby's status, your body, your timeline) and usually a range of choices — wait and monitor, schedule it, or go home and come back. You and your provider decide together.

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INDUCTION PRESSURE



“Thank you — let's talk through the timing. What happens if we wait two more hours? Two more days?”

When it comes up: the recommendation feels urgent, but nobody has explained what "urgent" means for you specifically.

Why it works: urgency is real sometimes, and vague sometimes. Asking for the timeline in concrete terms turns "we need to do this now" into information you can actually weigh. It keeps you collaborative while slowing the moment down.

What happens next: you'll hear what monitoring looks like while you wait, and what signs would change the plan. Most of the time you'll leave with a decision you helped make.

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INDUCTION PRESSURE



“This sounds like the standard protocol. Can we talk about what makes sense for my situation specifically?”

When it comes up: the recommendation arrives with urgency but without context, and you want it applied to **you**, not the average patient.

Why it works: it names the protocol without attacking it, then asks for personalization. Providers get this question constantly and good ones love it — it invites them to think with you instead of at you.

What happens next: your provider walks through your specifics. Sometimes the answer is still the protocol — and now you'll know why it's right for you, which is a completely different feeling.

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INDUCTION PRESSURE



“If we choose to start induction, what will the first steps look like, and how long is each one?”

When it comes up: you've decided to move forward and you want the picture before anything starts.

Why it works: agreeing and asking for the roadmap keeps you in the driver's seat even when you've said yes. You'll know what to expect hour by hour, which lowers the fear of the unknown — the loudest thing in most labor rooms.

What happens next: your nurse or provider maps the sequence (medication, monitoring, checks). You know what each step is for before it happens.

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INDUCTION PRESSURE



“We'd like a few minutes alone to talk about it before we decide. Is now a good time to have that space?”

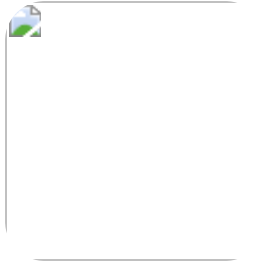
When it comes up: the room fills up with people and opinions and you haven't had a second to hear yourself think.

Why it works: it's polite, clear, and completely reasonable — and it's your right. Asking directly for space ("is now a good time") invites the team to support you instead of feeling dismissed.

What happens next: the team steps out, you and your partner (or you and your doula) talk, and you come back to the conversation ready. That pause is often where the real decision lives.

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2 · Epidural & Pain Relief Discussion

4 sentences for this moment.

EPIDURAL & PAIN RELIEF DISCUSSION



“I’d like to understand my pain options — including doing nothing medically for now. Can we go over them all?”

When it comes up: early in labor, or when pain relief comes up before you've thought it through.

Why it works: it puts *all* the options on the table — including waiting — without signaling a decision either way. It makes pain management a conversation, not a checkbox.

What happens next: you'll hear the full menu (epidural, other medications, breathing and movement tools, or a mix). Nothing is decided until you decide.

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EPIDURAL & PAIN RELIEF DISCUSSION



“What does getting an epidural involve right now — and what would happen if I waited a bit longer?”

When it comes up: the conversation is moving toward an epidural and you want the moment to slow down before you commit.

Why it works: it treats the epidural as a real choice with timing, not an automatic next step. Asking "what would happen if I wait" is a completely reasonable question that keeps your options open.

What happens next: you'll learn how the epidural works, how long setup takes, and what waiting does or doesn't change. Then you choose — with the facts, either way.

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EPIDURAL & PAIN RELIEF DISCUSSION



“I’d like to try a while longer without it, and I want you to know that could change. Can we agree that’s okay?”

When it comes up: you want to keep going unmedicated for now but you don't want anyone keeping score.

Why it works: it states your current choice AND pre-permits yourself to change your mind. Nobody has to guess where you stand, and both options stay open without judgment.

What happens next: your team knows the plan and knows it's flexible. You labor on your terms, and the door stays open both ways.

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EPIDURAL & PAIN RELIEF DISCUSSION



“The epidural is on the table and we’re considering it. Before we decide, can you tell us what it will and won’t change?”

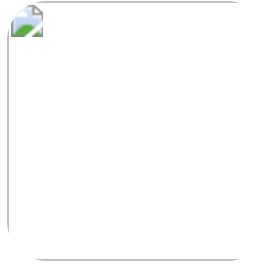
When it comes up: you’re genuinely torn and want the honest trade-offs, not a sales pitch in either direction.

Why it works: it asks for truth, not persuasion. A good care team will tell you exactly what changes (pain relief, mobility) and what doesn’t (the labor itself). You get to decide from reality.

What happens next: you hear the real picture and make the call that fits **your** labor. Both answers are fine; only one is yours.

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3 • Unexpected Intervention Recommendation

4 sentences for this moment.

UNEXPECTED INTERVENTION RECOMMENDATION



“That’s new information for us. Can you explain what’s changed and what the options are now?”

When it comes up: something shifts in labor (monitoring changes, your contraction pattern changes, or new information comes in) and the plan you prepared for suddenly isn't the plan anymore.

Why it works: it acknowledges the moment ("new information"), asks for the reason, and asks for options — without panic and without refusal. It's the sentence that keeps you a participant when everything speeds up.

What happens next: your team explains the change. You get the updated picture and the updated choices. The plan changes; your role in deciding doesn't.

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UNEXPECTED INTERVENTION RECOMMENDATION



“What are the risks if we do this, and what are the risks if we wait and monitor?”

When it comes up: a recommendation arrives mid-labor and you need to weigh it quickly.

Why it works: it frames *both sides* of the decision — doing something and doing nothing both have trade-offs. Asking about both makes you an informed decision-maker in under thirty seconds.

What happens next: you'll hear both pictures. Sometimes waiting is clearly right; sometimes acting is. Either way, the decision is now yours, made from both sides.

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UNEXPECTED INTERVENTION RECOMMENDATION



“I hear you, and I trust this is serious. Can we have a moment to absorb this and then decide together?”

When it comes up: the moment is heavy — a recommendation you didn't expect, delivered quickly.

Why it works: it does two things at once: accepts the seriousness (your team needs to know you're not ignoring them) and claims the space to think. Serious moments still deserve a breath.

What happens next: the team gives you the moment (they almost always do when asked directly), then returns for your decision. You'll make it present, not panicked.

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UNEXPECTED INTERVENTION RECOMMENDATION



“Is there a version of this we can try first that's less of a change from our current plan?”

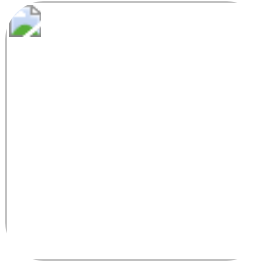
When it comes up: you want to meet the concern the team raised without jumping to the biggest step.

Why it works: it asks for intermediate options — the step between "nothing" and "everything." There very often is one, and it keeps your preferences in the decision instead of replacing them.

What happens next: your team either offers a middle path (great — take it and stay watched) or explains why there isn't one. Both answers are useful information.

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4 • Consent & Paperwork

4 sentences for this moment.

CONSENT & PAPERWORK



“Before I sign, can you explain what I’m agreeing to in plain language?”

When it comes up: a clipboard appears with a form and a quick "just sign here."

Why it works: "plain language" is the magic phrase — it tells them you want to understand, not recite. Consent you actually understood is consent that means something.

What happens next: they walk you through it in real words. If something doesn't make sense, ask again — you haven't held anyone up; you've done your job.

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CONSENT & PAPERWORK



“What happens if I don't sign this right now?”

When it comes up: the pace feels fast and the signature feels automatic.

Why it works: it's a completely fair question asked without accusation. The answer tells you whether this is truly urgent or just routine — and those are very different situations.

What happens next: you'll hear which it is. Most of the time there's more room than the pen-in-hand moment suggests, and you sign when you know what you're signing.

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CONSENT & PAPERWORK



“Can I have a copy of this to keep, and a few minutes to read it?”

When it comes up: any consent form, honestly — but especially ones that arrived mid-labor.

Why it works: reading your own paperwork is not an inconvenience; it's your right. Asking for a copy also signals you take it seriously — which makes the whole exchange more respectful.

What happens next: you get the form and the time. You read, you ask, you sign when you're ready.

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CONSENT & PAPERWORK



“Can someone go through the specific risks listed here with me?”

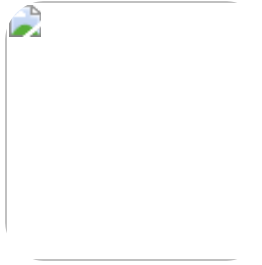
When it comes up: the form has a wall of fine print and nobody's explained it line by line.

Why it works: it turns fine print into a conversation and puts a real person in charge of explaining it. Nobody should consent to a list of words they can't explain.

What happens next: a nurse or provider sits with you and translates the list. You sign knowing what each item means for you.

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5 • Communication Breakdowns

4 sentences for this moment.

COMMUNICATION BREAKDOWNS



“I think we’re talking past each other. Can we back up — here’s what I understand, and here’s what I’m asking.”

When it comes up: you’ve asked something twice and gotten an answer to a different question.

Why it works: it names the breakdown without blame and then *rebuilds* — stating your understanding first makes it easy for them to correct exactly the part that’s off.

What happens next: the misunderstanding gets located and fixed, usually in one exchange. The conversation moves forward with both of you in it.

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COMMUNICATION BREAKDOWNS



“I’m hearing conflicting information. Can you help me understand how this fits together?”

When it comes up: your nurse says one thing, the doctor says another, and you're caught in the middle trying to make sense of it.

Why it works: it puts the *team* back in "care team" — you're not choosing sides or reporting anyone, you're asking them to reconcile their own picture. It's respectful and it works.

What happens next: someone takes ownership of the full picture and explains how the pieces fit. Sometimes this genuinely improves your care, not just your understanding.

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COMMUNICATION BREAKDOWNS



“This matters a lot to me and I want to make sure it doesn't get missed. Can we come back to it?”

When it comes up: you've said something that matters and it's sliding off the conversation.

Why it works: it's direct without being adversarial — "I don't feel heard yet" is an honest report, and "can we try once more" is an invitation, not an accusation. It's hard to ignore and impossible to be offended by.

What happens next: most providers reset and truly engage. You get your point back into the conversation, where it belongs.

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COMMUNICATION BREAKDOWNS



“Could you write down the key points, or can my support person take notes while we talk?”

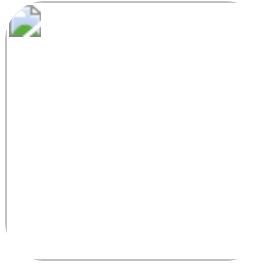
When it comes up: the information is coming fast and you know you won't remember it accurately.

Why it works: it normalizes documentation in the room. Notes protect *you* (memory, decisions) and *them* (clear understanding), so both sides usually welcome it.

What happens next: the details land on paper. Afterward you can reread, and your support person can help you process what was said.

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6 • Scheduling & Pace Pressure

4 sentences for this moment.

SCHEDULING & PACE PRESSURE



“I understand there's a schedule to keep. What's the medical reason for that timing in my case?”

When it comes up: things are moving on the ward's rhythm — and you want to know how much of it is medicine and how much is logistics.

Why it works: it separates medicine from logistics politely. Hospitals have real schedules; asking which reasons are medical puts the focus back where it belongs.

What happens next: you'll get the medical picture — and the honest parts about logistics. Then you decide with clear eyes about what's driving what.

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SCHEDULING & PACE PRESSURE



“We're not ready yet, and we're comfortable with where things are. Can we have more time?”

When it comes up: you're being asked to move to a next step and your gut says not yet.

Why it works: it's simple, warm, and complete — states where you are, states the request, asks for nothing dramatic. "Not ready yet" is a legitimate answer in almost every birth conversation.

What happens next: usually, you get time. Sometimes you'll hear a reason why waiting matters — and that's information worth having, offered in the same calm register.

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SCHEDULING & PACE PRESSURE



“If we wait another hour, what would we be watching for that would change our minds?”

When it comes up: you want to buy time *and* stay safe — the smart combination.

Why it works: it turns waiting from passive into active. You're telling the team: we'll wait, and we want to know the specific signs that would make waiting the wrong call. That's what collaboration looks like.

What happens next: the team defines the watch-points with you. Time passes with everyone watching the same things — which is exactly how waiting should work.

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SCHEDULING & PACE PRESSURE



“Can we plan for the rest of today together, so we’re not deciding each step under pressure?”

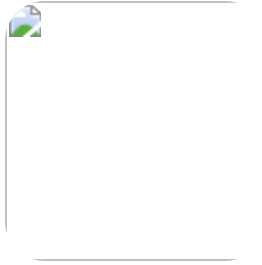
When it comes up: the day has become a chain of small decisions, each one arriving faster than the last.

Why it works: it lifts the conversation from step-by-step pressure to a shared map. Providers generally prefer this too — a patient who wants to plan the day is a patient they can actually work with.

What happens next: you walk the day's likely path together, including the decision points. When each step arrives, it's familiar instead of sudden.

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7 • Being Alone or Unsupported in the Room

5 sentences for this moment.

BEING ALONE OR UNSUPPORTED IN THE ROOM



“I’d like my support person (my doula/partner) in the room for this conversation. When can they come back in?”

When it comes up: a shift change, a procedure, or a hallway conversation is happening and your person got left outside.

Why it works: it frames your support person as part of the team — because they are. Asking “when can they come back” treats their absence as temporary logistics, not a policy.

What happens next: your person rejoins you, usually quickly. Support in the room changes every conversation that follows.

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BEING ALONE OR UNSUPPORTED IN THE ROOM



“I’m feeling alone in this and I need some support right now. Can someone stay with me?”

When it comes up: the room is quiet, everyone's at the desk, and you're doing the hardest work of your life by yourself.

Why it works: asking directly for support is strength, not weakness — and it gives the team something specific and doable. Nurses want to know this. Most will stay, or send someone who can.

What happens next: someone stays with you, checks in, or explains when they'll be back. You're not doing this alone anymore.

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BEING ALONE OR UNSUPPORTED IN THE ROOM



“Before any procedure starts, can you make sure my support person is here with me?”

When it comes up: something is about to happen and you want your people beside you for it.

Why it works: it makes your support person's presence a standing part of the plan rather than something you renegotiate each time. One sentence, stated once, respected all day.

What happens next: the team builds the wait-for-my-person into the routine. You go into every procedure with your people.

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BEING ALONE OR UNSUPPORTED IN THE ROOM



“Who will be in the room for the birth itself, and can we talk about who I want there?”

When it comes up: the room's population is growing and you haven't been asked.

Why it works: it asks for the guest list in advance — a completely normal request that's easy to forget you're allowed to make. Your birth, your room.

What happens next: you find out who's planning to be there and you get a real say. Most teams honor this fully once it's voiced.

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BEING ALONE OR UNSUPPORTED IN THE ROOM



“Can you stay while we talk this through? I do better when I can ask questions in the moment.”

When it comes up: someone important is about to leave the room right when you need them.

Why it works: it gives the provider a reason to stay that costs nothing. "Questions in the moment" is how good medical conversations work — you're just asking for the good version.

What happens next: they stay, you talk, and the decision happens in one complete conversation instead of fragments.

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These sentences work best inside a real plan.

The Joyful Birth Plan is our free, fillable birth plan — it shows you where each sentence fits, before you ever walk through the doors.

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Go deeper: Birth Plan Confidence Session — practice these 1:1 with Shelbi → truejoybirthing.com

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